# Trauma-Informed Approaches to Campus Sexual Assault Response: A Mixed-Methods Study

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**ABSTRACT**

This study examined attitudes towards and implementation of trauma-informed approaches in sexual assault response protocols at U.S. higher education institutions through a two-phase mixed-methods approach. This included surveying 830 institutions and interviewing 43 participants representing seven of those institutions. Staff involved in response efforts supported trauma-informed approaches, but were often constrained by federal requirements (i.e., the Clery Act and Title IX), and experienced difficulty managing different priorities across a response team. Study results demonstrated the need for improved translation of federal rules into practical guidance for sexual assault responses and indicated collaboration was a key facilitator for successful, student-centered responses.

**Keywords**: sexual assault; higher education; Title IX; trauma-informed

## Introduction

Sexual assault at institutions of higher education (IHEs) is a pressing issue, as demonstrated by the significant numbers of students affected by sexual violence and the resulting short and long-term health consequences. Studies estimate that 20-33% of women report experiencing sexual violence since entering college (Conley & Griffith, 2016; Koss et al., 2022; Mellins et al., 2017). Men report sexual violence at lower but still notable rates, and students identifying as lesbian, gay, bisexual, transgender, or gender non-conforming report rates up to five times higher than their heterosexual and cisgender peers (Coulter et al., 2017; Eisenberg et al., 2021; Mellins et al.,

2017). Sexual assault prevalence also depends based on a confluence of different identities: the intersection between race, gender identity, and sexual orientation can predispose individuals to even higher risks of victimization and subsequent health consequences (Coulter et al., 2017; Sigurvinsdottir & Ullman, 2016).

Sexual violence has consequences on personal well-being, academic success, and relationship-building (Schroeder et al., 2023). Victimization results in significantly higher levels of self-injury, suicidal ideation, depression, anxiety, and post-traumatic stress (Khadr et al., 2018; Parr, 2020; Rothman et al., 2021). Victimization affects academic progress (e.g., declining GPAs, decreased motivation, taking time off) and interpersonal relationships (e.g., decreased emotional and sexual intimacy in romantic relationships) (Potter et al., 2021; Rothman et al., 2021). These outcomes are observed immediately after an event and may persist for years afterwards (Khadr et al., 2018; Potter et al., 2021; Rothman et al., 2021).

Evidence Base For Trauma-Informed Approaches

Experiencing trauma, including incidents of sexual violence, can have lasting, adverse effects on an individual's mental, physical, social, emotional, and spiritual well-being (SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach, 2014). There is growing recognition that systems lack capacity to respond to people experiencing trauma, especially sexual assault survivors (Fernández et al., 2023). Immediate medical and law enforcement responses can cause additional trauma through victim-blaming behavior and questioning, a lack of belief in the report, and requiring the reporter to re-tell their story numerous times (Brown et al., 2022; Maier, 2008). Care after an assault, including the chaotic nature of an emergency department, the patient-provider power dynamic, and the invasiveness of forensic exams, can also traumatize (Brown et al., 2022; Reeves & Humphreys, 2018).

Trauma-informed approaches (TIA) gained traction in the late 1990s as a method to address sexual assault incidents and similarly traumatic events (Fernández et al., 2023). SAMHSA defines a trauma-informed approach as one that "*realizes* the widespread impact of trauma and understands potential paths for recovery; *recognizes* the signs and symptoms of trauma in clients, families, staff, and others involved with the system; and *responds* by fully integrating knowledge about trauma into policies, procedures, and practices, and seeks to actively *resist re-traumatization*" (SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach, 2014). SAMHSA also outlines six principles to further a trauma-informed approach, which are described in Table 1. Constructively, TIA have a strong evidence base from studies of the child social services system, where robust interventions have increased implementation of TIA and resulted in reduced posttraumatic symptoms and behavior issues in children who have experienced abuse and neglect (Bartlett et al., 2016; Jankowski et al., 2019; Lang et al., 2016).

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Table 1 about here

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While there are fewer studies examining the efficacy of TIA for responding to adult sexual assault survivors, there is evidence of its potential in health care, law enforcement, and IHE settings. TIA are currently implemented through Sexual Assault Nurse Examiner (SANE) programs, rape crisis centers, and victim advocacy organizations, and have demonstrated improved outcomes for sexual assault survivors (Campbell et al., 2012). One recent study showed that nurses who underwent a trauma-informed curriculum for forensic sexual assault care significantly improved their attitudes and knowledge relevant to TIA and their ability to practice trauma-informed techniques (Nathan & Ferrara, 2020). There are fewer efforts to implement TIA in law enforcement settings (E. Lathan et al., 2019; Rich, 2019), but a growing body of literature

demonstrates the positive impacts trauma-informed training can have on officers' ability to interact with survivors of sexual assault. Trained officers had greater understanding of trauma symptomology, personal investment in learning about sexual assault, and improved victim-interviewing skills (E. Lathan et al., 2019; E. C. Lathan et al., 2022; Rich, 2019). At IHEs, there are some efforts to help staff recognize symptoms of trauma (resulting from sexual assault and other events) so they can respond in an empathetic and trauma-informed way (Hunter, 2022; Wood et al., 2021; Yoshimura & Campbell, 2016).

Policy Barriers For Implementing TIA At IHEs

IHEs' compliance with federal policies, namely the Clery Act and Title IX, can conflict with TIA principles during sexual assault responses (Fisher & Cullen, 2013; Streng & Kamimura, 2015). The Clery Act, passed in 1990, requires IHEs to submit annual reports to the Department of Education (DoE) disclosing campus safety policies, procedures for sexual assault prevention, and resources provided to crime survivors. The Clery Act intended to incentivize IHEs to report, investigate, and respond to violent crimes; however, campus administrators have commented on the difficulties of complying with the law and its incompatibility with TIA. Staff struggle with the focus on data collection for compliance (rather than crime prevention or student support), managing dual campus disciplinary and law enforcement processes (and the negative impacts resulting from confusion over these reporting streams), and reconciling federal, state, and institutional policies (Fisher & Cullen, 2013; Terman, 2022).

Title IX was passed in 1972 to ensure male and female students were treated equally and fairly in educational settings and to encourage IHEs to prevent and respond to sexual violence on their campuses (Streng & Kamimura, 2015). Over the last few presidential administrations, Title IX policy reforms have been both praised and criticized for advancing or conflicting with trauma-informed principles. In 2011 and 2014, the DoE Office of Civil Rights under the Obama administration issued two Dear Colleague letters (DCLs), directing colleges to respond to sexual assault cases in a more survivor-centered way. Key changes included allowing a lower evidence bar for adjudicating cases (the "preponderance of evidence" standard); permitting a single investigator model (allowing one person to investigate misconduct and decide case outcomes); and discouraging live hearings and cross-examination for resolving cases (Mancini et al., 2023; Melnick, 2020). The guidance also encouraged institutions to provide support services to survivors of violence, such as medical advocacy (e.g., information about rape kits), educational accommodations, safety planning, and information about pursuing legal recourse (Conley & Griffith, 2016). However, the 2011 DCL also required that non-confidential university employees report incidents of sexual violence to the Title IX coordinator, thus establishing mandatory reporting policies for many staff (Mancini et al., 2016; Newins et al., 2018). While this requirement was intended to improve campus awareness and response to sexual violence, it is criticized for limiting students' agency in how broadly they share their report of sexual assault—potentially putting their safety at risk—and breaching trust between students and staff (Weiss & Lasky, 2017).

The first Trump administration revoked the Obama-era policy guidance through a 2017 DCL and updated Title IX rules in 2020 (Mancini et al., 2023). The new rules permitted a more stringent evidence standard to be used in resolving cases ("clear and convincing," rather than "preponderance"), required live disciplinary hearings with the option of cross-examining parties, prohibited the single investigator model, and prevented schools from placing interim restrictions on a respondent prior to the case's decision (Melnick, 2020). The introduction of live hearings with cross-examination and the prohibition of additional accommodations to complainants (that might advantage them over the respondent) have been observed as constraining campus staff in

fulfilling the trauma-informed goal of providing physical and emotional safety to survivors of violence (Melnick, 2020). The Biden administration, through its Title IX rulemaking, reversed many of the changes introduced by the Trump administration (*Biden's New Title IX Rule*, 2024; Eckes et al., 2022).

However, shortly before the end of the Biden administration, a federal judge struck down the April 2024 final rule the administration had promulgated with its Title IX policies (Blinkley, 2025a). And at the start of the second Trump term, the Department of Education again directed institutions of higher education to return to the guidance issued under the first Trump administration (Blinkley, 2025b). Given the significant cuts to many federal departments and agencies, including nearly 50% of the staff cut from the Department of Education as of March 2025, it is unclear how campus sexual assault response efforts will be addressed throughout this second administration and beyond (*U.S. Department of Education Initiates Reduction in Force*, 2025).

These relatively rapid changes in federal policy have occurred concurrently with the advances in expert guidance on TIA and a reinvigorated focus on addressing sexual violence at IHEs (Henshaw, 2022). The #MeToo movement accentuated the pervasive nature of sexual violence across many settings, including IHEs, and galvanized student community and institutional efforts to prevent sexual assault and support survivors (Gronert, 2019). As these cultural and legal changes have shaped conversations about IHE sexual misconduct response protocols over the past decade, they have also raised questions about how these are implemented in different campus settings. A large body of research covers the magnitude of the issue and the difficulties students face as a consequence of experiencing sexual violence (Fedina et al., 2018; Klein & Martin, 2021). While there is some evidence that TIA have permeated into IHE sexual misconduct response efforts, literature is limited to small-scale efforts within health centers, studies identifying barriers, or commentaries calling for greater implementation of the practice in the higher education setting (Hunter, 2022; Yoshimura & Campbell, 2016).

## The Present Study

The Campus Sexual Assault Response (CSAR) study was designed as a mixed-methods study seeking to understand the extent to which TIA have been adopted and implemented within a nationally representative sample of U.S. IHEs in response to sexual misconduct. We posed three main research questions: (1) How do CSAR teams implement TIA when responding to sexual assault?; (2) How does the implementation of TIA differ across sectors (i.e., health and wellness ["health"], safety and security ["safety"], Title IX and compliance, and student services)?; and (3) How do current regulatory constraints affect IHE staff in their efforts to implement TIA? The quantitative results provided answers to our first two research questions, gathering data about how CSAR teams implemented TIA in response to sexual assault reports, and how TIA implementation differed across sectors. Qualitative data elaborated on all three of our research questions but specifically informed the question about how regulatory constraints affect CSAR teams implementing TIA.

## Methods

This two-phase mixed-methods study used quantitative and qualitative methods to capture data from sectors involved in CSAR activities at IHEs. Phase I consisted of a nationally representative survey of IHEs, fielded from October 2020--August 2021. Phase II involved key informant interviews (KIIs), conducted between February--May 2023, to gather additional information on CSAR activities from a subset of seven IHEs that participated in the Phase I survey. Henceforth, we will refer to survivors of sexual violence as "complainants" and accused perpetrators as "respondents," to be consistent with the language used by IHE administrators.

## Quantitative Methods

### Survey Data Collection

We constructed a nationally representative sample of IHEs using the 2018 DoE Integrated Postsecondary Education Data System (IPEDS) as our sampling frame. We selected 1,500 IHEs, stratified by type of institution (two- vs. four-year; public vs. private) and student body population size, with weights to adjust for selection probability and nonresponse. The survey consisted of two modules: one offered to campus safety staff (Module 1) and the other to health staff (Module 2). Selected IHEs were contacted with an online screener for the identification of health/wellness and public safety points of contact. These points of contact received links to Modules 1 and 2 directly. Directions for each module included encouragement for recipients to consult or share the survey link with colleagues who would be best able to answer the survey questions for their sector at their IHE. The current paper presents survey findings from 830 campuses that completed Modules 1 and 2 to facilitate comparisons between sectors.

### Measures

Staff Categories Within Sectors**.** Within each module, participants were asked to specify what categories of staff work within each sector on campus. For safety participants, response options included contract security, institutional public safety (non-sworn), institutional police (sworn, including peace officers), and external law enforcement. Within the health module, participants were asked what services students could access by visiting a physical space on campus. Response options included health care services (e.g., diagnosis, treatment, and other clinical care), health promotion services (e.g., health education, community events, policy review, and other wellness services), mental health services (e.g., counseling and psychological care), and other victim support services (e.g., victim advocates for students reporting sexual assault).

Trauma-Informed Skillsets. All safety staff and select health participants were asked the degree to which they agreed their staff members were experienced in four trauma-informed skillsets (1=strongly agree; 5=strongly disagree). These skillsets included (1) the ability to recognize signs of trauma (asked of health care, mental health, and victim support staff); (2) advocating for TIA when responding to sexual misconduct (asked of health promotion and victim support staff); and implementing strategies both (3) immediately and (4) over time to support students as they recover from trauma related to sexual misconduct (asked of health care, health promotion, mental health, and victim support staff). Responses were re-coded to a 3-point scale, combining the “strongly agree” and “agree” responses, and the “strongly disagree” and “disagree” responses.

### Survey Data Analysis

We conducted descriptive analyses summarizing the responses to the skillsets questions across staff categories within each sector and differences between sectors. We used survey weights, accounting for the probability of selection and nonresponse, in all analyses (conducted with SPSS 29.0), such that our estimates are nationally representative.

## Qualitative Methods

### Key Informant Interview Data Collection

We constructed a list of campuses that completed both modules of the survey, stratified by campus type (public, private, minority-serving institution), size, and degree program (2-year, 4-year), aiming to capture detailed information on CSAR activities from a range of perspectives. We also aimed for geographic diversity by region of the U.S. and other demographic characteristics (e.g., percentage male vs. female, majority/minority white). We invited a sample of campuses to participate and worked directly with campus contacts to identify appropriate staff participants who could shed light on their CSAR activities, with a requirement that each IHE

include at least one representative from the health and safety sectors.
We conducted KIIs with 43 participants across 7 campuses, with 5-7 participants per campus. Table 2 outlines the characteristics of our sample institutions and the roles of the interviewees at each institution. Our sample included campuses from the northeastern, southeastern, midwestern, and southwestern regions of the U.S. Some campuses had a single location, while others had locations across a metro area or region. Schools varied by the percentage of the student population that lived on-campus, which had implications for the robustness of residence life services, presence of on-campus police, and policies for handling off-campus sexual harassment and assault. In addition to health and safety, KII participants also represented Title IX and Compliance offices, victim advocates, and student services (e.g., residence life). Notably, the KII sample included some staff who were confidential resources and were thus *not* required to share information about reported sexual misconduct with Title IX, and other staff who were mandatory reporters and required by law to share such information with their Title IX office.
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Table 2 about here
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We created KII guides covering the same broad questions, with each slightly tailored to the interviewee type (e.g., health, safety, other sectors) through specific probes. The guides covered collaboration and communication during incident response and typical day-to-day interactions. The guides also asked participants about their personal views towards TIA and how these were implemented at their institution. However, these questions were intentionally asked at the end of the interview so we could assess the extent to which participants organically discussed TIA before being asked about it explicitly.
Interview Data Analysis
Interviews were transcribed verbatim, de-identified prior to analysis, and coded using MAXQDA 2022 (release 22.2.1). We used a deductive thematic analysis, beginning with developing a deductive codebook designed to capture information on TIA, how participants work independently or collaboratively to address sexual assault, and facilitators and barriers to CSAR activities (Hennink et al., 2020). We coded data in two rounds. In the first round, the coding team independently coded all transcripts to capture broad themes, including *any* references to TIA. In the second round, the first author reviewed all data coded as related to TIA and applied a set of codes based on the six SAMHSA principles for implementing TIA, as outlined in Table 1. Data were double-coded when applicable to capture relationships between the SAMHSA principles. After the second round of coding, we synthesized themes within coded data to understand how SAMHSA's principles were enacted in sexual assault responses at IHEs. We ran queries to identify patterns in the six principles within each campus, different staff categories (i.e., themes within all health staff and all safety staff), and as they intersected with other principles.
Results
Trends Across Sectors
Figure 1 presents the weighted proportion, by staff category, of participants who agree that most staff have four skillsets they can employ when interacting with students who have experienced trauma related to sexual misconduct.
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Figure 1 about here
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Comparing responses across the four skillsets, both safety and health participants showed the highest levels of agreement, across all staff categories, with the statement that most staff "are

able to recognize the signs of trauma in students" (with participants indicating that at least half of all staff possess this skill). Participants were relatively less certain that "most staff advocate for trauma-informed responses" (with only 27% of contract security staff possessing this skill). Participants from both safety and health sectors demonstrated more confidence in their respective staffs' ability to implement strategies to support students *immediately* as they recover from trauma related to sexual misconduct compared to their ability to implement strategies to support students *over time*.

When looking across staff categories, staff from the health sector reported higher agreement that their staff leveraged trauma-informed skillsets than staff from safety sectors. Within the safety sector, participants were less likely to agree that contract security officers have these skillsets. Within the health sector, victim support and mental health staff had the highest ratings for these skillsets. In particular, victim services staff had the highest ratings for TIA skillsets across all four quantitative measures.

While the quantitative results provided a nationally representative snapshot of how TIA are viewed and practiced across a varied range of campuses, regions, and staff roles, qualitative data were essential to illuminate the nuances of these practices. Qualitative analysis revealed that participants across sites and sectors (health, safety, Title IX and compliance, student services, advocacy) were generally supportive of TIA. They expressed awareness of the widespread nature of trauma within their student communities and described how ignoring the impacts of trauma could lead to consequences for students, such as decreased engagement in academics, poor mental health outcomes, and potentially dropping out of college. They described trainings conducted on campus to promote TIA, including trainings on how to recognize and respond to trauma in students.

### Facilitators For Implementing TIA

The qualitative results were essential for illuminating specific practices used by IHEs to implement TIA. SAMHSA specifies six principles of TIA, which should be applied when responding to trauma to achieve the best outcomes and avoid additional trauma (SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach, 2014). Below, we describe how participating campuses put each principle into practice on CSAR teams.

#### Empowerment, Voice, Choice

Participants described empowering students by giving them agency about what they share and with whom, allowing them to choose their next steps in the incident response process, and limiting the number of times a student had to re-share their story, if possible. Because Title IX and criminal investigations into a reported sexual assault can lead to additional trauma for a student, another key aspect of this principle was providing students with their available options and possible outcomes of each path to facilitate their informed decision-making. One safety representative discussed a process through which a student could report without committing to a further disciplinary investigation: "We are able to arrange a meeting with the police in which everything that is said there is in a hypothetical sense. [...] If at the end of this interaction the complainant […] says that they don't want to do anything further, that conversation never happened" (Site 1, Safety & Security). If a student first disclosed to a confidential resource, that staff member would clarify next steps depending on whether the student wanted to report to the institution, or limit their disclosure to confidential resources.

To be immediately prepared to respond to students, participants recommended maintaining a solid knowledge of the available resources (e.g., counseling, advocacy, law enforcement, cultural centers) and the different reporting processes and associated implications for a student if they shared their story with certain staff (i.e., mandatory reporters versus confidential resources). For

example, one participant developed a flow chart to outline the possible outcomes and risks of a student's choices for reporting and recourse in an easily digestible format. Participants emphasized "putting a student in the driver's seat" by respecting a student's wishes for moving forward, avoiding pressure on the student to choose one path over the other, and remaining flexible if a student changed their mind. One advocacy staff member said, "I will provide you with options. I'm never going to tell you what to do. You are the expert in currently what is happening in your life, not me. I have tools in my toolkit that can help you [...] or help you feel supported" (Site 2, Advocacy).

### Collaboration And Mutuality

Participants emphasized the importance of timeliness, sensitivity, and good communication when responding to a sexual assault report. To achieve these goals, many participants discussed building relationships with each other by meeting regularly, debriefing past case responses, discussing their goals and priorities, and maintaining friendly working relationships with each other. One participant noted, "Good collaboration is getting the resources to the student or faculty involved. [...] Working together if, e.g., a victim wants to change housing, how quickly can we do that, how can we minimize steps they need to go through; everyone should be familiar with policies, how to do everything effectively and quickly" (Site 7, Title IX & Compliance). Being flexible and student-centered were facilitators for working well with others; it was helpful when staff were not territorial about their work and demonstrated shared willingness to work beyond minimum job requirements to promote students' well-being. One participant explained, "I know each of us has kind of a different specialty, […] like public safety and police, they have their own focus, and I have my focus, and Title IX has their own focus. But I think we do a pretty good job of making sure that students know what their resources are" (Site 4, Health & Wellness).

### Trust And Transparency

Participants reported different methods for building trust with students. They emphasized cultivating trust prior to incidents by maintaining a presence in the community and developing friendly relationships with students. For example, one participant noted, "I don't think that you can wait until an incident happened to try to start building relationships with the students so that they know that they can trust you, they believe in you, which will encourage them to report these types of incidents" (Site 4, Safety & Security). Regarding incident response, participants described validating the student's side of the story, actively listening, and explaining how they could leverage their role to help the student. To promote transparency, participants found it helpful to lay out a student's options for moving forward, describe the full process and potential outcomes for each option, and clarify the scope of their role and how they could provide assistance.

### Safety

Participants practiced TIA through collaborating with students on safety planning. This included assisting students in navigating systems at the institution, creating physical safety routes on campus, and working with students to develop coping skills to respond to possible triggers. Participants also described techniques for creating an emotionally safe environment in the immediate moment a student reported a sexual assault by observing a student's body language, listening without interrupting, emphasizing they recognized the sensitivity of the incident, and not requiring the student to re-tell their story multiple times. One participant described how they created safety with a specific student, saying, "[The complainant] thanked me just for basically listening to her and not judging her for anything and was just like, 'I just feel better knowing that I can come and talk to someone and things like that.' […] It's about creating that safe space and

giving the person that opportunity to speak freely and listen in every step of the way" (Site 3, Student Services).

### Peer Support

The principle of peer support was not raised as often by participants as other principles of TIA, but a few participants discussed ways to facilitate peer support for students who experience sexual assault. Some campuses convene group therapy sessions for survivors or peer education groups aimed at sexual assault prevention. One staff noted, "We recently just started a survivor support group, and that's been incredibly impactful. […] The first thing people always said to me was, now that I've done this horrible process, I want to help other people. […] But actually, what also happens is that, like, personal healing" (Site 6, Advocacy). Interview participants acknowledged how the peer network is important in sexual assault response because students will often initially seek help from friends, and described efforts across their campus to train students as peer educators or counselors.

### Cultural and Gender Issues

There were comparatively fewer data for this principle, but staff shared examples of how they sought to understand the diverse backgrounds of their student population and respond with empathy during response efforts. Participants recommended learning about the student populations who may have greater needs (e.g., recognizing when there are higher proportions of first-generation or low-income students in the student population) and tailoring response efforts to these groups. One participant elaborated, "There are just many, many layers of […] what can trigger people's trauma responses. And so, for me, I think the biggest thing is really just trying not to have any assumptions. […] I try not to formulate too much of a plan before I see people" (Site 1, Health & Wellness). Participants also described the importance of training staff to strengthen culturally competent student engagement, such as by providing education about transgender identity, using individuals' preferred pronouns, and understanding the differences between gender, sex, and sexual orientation.

### Relationships Between Principles

As a result of our double-coding of qualitative data that represented more than one principle, our analyses captured the overlapping nature of the six principles. We found that four were especially closely related to one another – wherein the strategies to implement one principle helped bolster another. Figure 2 summarizes the main relationships between these four: empowerment, voice, and choice; collaboration and mutuality; trust and transparency; and safety.

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Figure 2 about here

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## Barriers to Implementing TIA

Despite collective support for TIA, quantitative results and qualitative interviews demonstrated barriers to implementing the six principles. These challenges were due to federal policy constraints, having to manage multiple goals and priorities across members of response teams, and a lack of resources.

### Federal Policy Constraints

A lack of compliance with Title IX and the Clery Act can prompt federal investigation or fines for violations, which may influence staff to interpret guidance and implement regulations with caution (Conley & Griffith, 2016; Mancini et al., 2016). Staff responsible for ensuring adherence to these laws, specifically Title IX and safety sector staff, frequently reported constraints related to these policies when implementing TIA. This reflects trends from the quantitative results, where victim advocacy staff demonstrated the highest skillset in using TIA when responding to

sexual assault, especially as compared with safety staff.

Title IX staff were bound by regulations that conflicted with their efforts to promote empowerment and safety. For example, at the time of the study, Title IX regulations required a live hearing and cross-examination to adjudicate a case. Participants noted this policy intimidated students and served as a tangible deterrent to a student going forward with the Title IX resolution process. One participant said, "When you let a student know that [you can pursue] an informal resolution or you have to go through a live hearing and get cross-examined by an advisor of [the respondent's] choice, no 18-year-old, 19-year-old student wants to subject themselves to that" (Site 4, Title IX/Compliance). An advocate added that their campus administrators interpreted federal policy as precluding victim advocates from being physically present during a Title IX hearing to provide emotional support and comfort.

Title IX's mandatory reporting requirements were also characterized as barriers to ensuring a student complainant experienced trust, empowerment, and safety. Staff who shared about their mandatory reporting responsibilities with a student prior to disclosure might stop the student from sharing. In other cases, staff who were mandatory reporters might not have the chance to share their status with students prior to their disclosure, which limited the student's choice of how widely to share their story and their privacy. IHEs attempted to mitigate these mandatory reporting difficulties by training faculty, staff, and student resident assistants about their responsibilities and encouraging mandated reporters to have candid, upfront conversations with students who may turn to these individuals to report. These training measures were meant to avoid situations where students disclosed to staff members they believed to be trusted relationships, without notice that the staff member was responsible for escalating the report. While these processes prevented students from feeling shocked and betrayed by the staff member's responsibilities, they also came with the cost of chilling conversation. One participant said, "I want conversation to feel organic with students. And when I say upfront, 'Here's what I can't keep to myself,' sometimes I'm wondering, are they not telling me something that they normally would have?" (Site 5, health).

Some staff also noted that Title IX's role may require them to override the wishes of a student who does not want to investigate their case, if they need to take action on a broader threat to the campus community. These staff perspectives may help explain why the quantitative data showed victim advocates—who are not mandatory reporters—were the staff group most likely to implement strategies to support students immediately and in the long-term as they recovered from trauma. Victim advocates' ability to keep confidentiality, thereby maximizing a student's trust in them and sense of empowerment, most aligns with TIA best practices.

Campus safety staff were also bound by the Clery Act requiring campuses to make their community aware of safety events. One participant noted how implementing this policy can negatively impact the campus's response and the complainant's safety. News of the incident, as it quickly spreads across the campus community, could frighten the complainant, introduce the risk of retaliation, and taint investigative efforts with gossip and rumors.

Conflicts Of Roles, Goals, And Priorities

Many challenges to implementing TIA resulted from differences in roles and subsequent priorities of team members in a response. Title IX and compliance staff were among the most enthusiastic supporters of TIA. They described themselves as regularly practicing TIA principles, even leading TIA trainings for other sectors on campus.

However, staff designated as confidential resources, such as counselors and advocates, indicated how they viewed Title IX and compliance staff roles as conflicting with the principles of TIA. They critiqued campus Title IX offices as having structural incompatibilities with trauma-

informed practices, due to their responsibility to support both complainants and respondents. These confidential resources suggested Title IX staff members at IHEs could improve communications and transparency to all parties throughout the incident response process in order to better incorporate TIA into their work. These participants also described strategies they used to empower students to take back their agency and choice, especially in circumstances where Title IX was required to take action regardless of the student's wishes. For example, these participants counseled students to refrain from answering questions that made them uncomfortable, or to simply not respond to Title IX's outreach if they did not want to pursue resolution.

Participants also described how the work of campus safety staff could be at odds with TIA principles. Because some campuses had both off-and on-campus police responding to initial reports of campus sexual assault within their jurisdictions, cases occasionally transferred between the different forces. This disrupted relationship-building between a student and the responding police, and had the potential to cause additional trauma for a student who had to repeat their story multiple times. Relatedly, quantitative data indicated external law enforcement were notably less skilled in TIA than institutional police. These findings suggest that external police forces who are not connected with institutional sexual response teams may be less well-versed in TIA, or less able to practice TIA with student complainants they encounter due to their limited integration with the campus community. The presence of external law enforcement in response activities could therefore exacerbate the potential for relationship disruption and additional trauma.

Participants also discussed how the role of campus safety clashed with the principle of empowerment, voice, and choice. While confidential resources like advocates and counselors talked about trusting complainants to make decisions that were best for them, security staff described navigating a difficult balance between protecting campus-wide safety and respecting the wishes of a student. For example, a student may want to continue with their academics and not pursue a full investigation, but the campus safety team is faced with a potential offender who could victimize other students on campus.

Some campus safety participants also suggested others in their own sector may not be on the same page about implementing TIA when interacting with a complainant, due to a lack of training in trauma-informed interview techniques and adherence to traditional police practices about how to look for deception. This was clearly represented in the quantitative data, which showed that contract security and institutional public safety staff had the lowest engagement in TIA across most skill measures. The TIA skills within these groups were often starkly lower than those of other safety roles.

Coordination Challenges

Across all sectors involved in response efforts, common challenges were related to the nature of collaborating with other members involved. This included a lack of streamlined coordination, such as multiple entities reaching out to a complainant and overwhelming them with resources. Poor communication and limited information sharing across responding staff could also lead to the student needing to repeat their story multiple times. A lack of timeliness could leave students feeling unsupported by the institution. Confusion about proper protocol for reporting could result in safety challenges for the entire campus, such as when staff failed to elevate information about a potential threat to the appropriate authority in a swift manner. A lack of adherence to campus policy could lead to an unsafe situation for a student (e.g., when staff directly reached out to a student thought to be in a domestic violence situation instead of reporting it to Title IX, who is trained to send discreet messages to students in need).

Insufficient Resources

Participants also described resource constraints limiting TIA, including time, personnel, and financial resources. Some participants raised the difficulty of establishing and maintaining peer support spaces, especially for staff who have many other job responsibilities. A reported barrier to promoting understanding of cultural and gender issues in response efforts was a lack of appropriate resources and staff to connect with students in a culturally competent way, such as an absence of staff of color who could relate to the lived experience of students of color. Another example was how socioeconomic disparities between students could appear during the Title IX hearing process, if one student had the resources to hire an external attorney but the other student could only afford to have their designated faculty member represent them.

## Discussion

This study describes how IHEs support students who have experienced sexual assault through the implementation of trauma-informed approaches in response efforts. Quantitative results indicated that health staff were generally better informed about and more likely to implement TIA in response to a sexual assault than safety staff. However, within safety staff categories, there was significant variation in reported TIA skill across different types of roles: for example, contract security staff had the lowest skill in TIA across all staff types in the study, whereas institutional police showed comparable levels skill in TIA to some health staff. This finding was supported by the qualitative findings, where some safety participants noted that not all safety staff were as well-versed in TIA. These findings reflect the limited but emerging role of TIA in law enforcement trainings, and points to an opportunity for greater police engagement in TIA when responding to reports of sexual violence (E. Lathan et al., 2019; E. C. Lathan et al., 2022; Rich, 2019).

Across both health and safety, survey participants were more confident in implementing TIA immediately after an event, as opposed to in the long-term, indicating the need to address the well-documented latent consequences of sexual violence (Khadr et al., 2018; Potter et al., 2021; Rothman et al., 2021). In addition, participants across sectors were less likely to agree that *most* staff they worked with advocated for trauma-informed responses as compared to other trauma-informed skills, echoing a sentiment in the literature that TIA may require greater promotion specifically in the IHE setting (Hunter, 2022). Lastly, across all staff types, victim services staff were rated as having the highest skill in TIA across all quantitative measures, a finding that was illuminated by interview participants indicating the training and nature of this role facilitated implementing TIA as compared with the training and nature of other campus staff roles. These findings point to the essential role in maintaining a TIA that victim services staff play on IHEs, though our survey data show they are underrepresented in number as compared to health and safety staff.

Qualitative findings illuminated facilitators and barriers to implementing TIA as defined by SAMHSA (SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach, 2014). Staff emphasized sharing institutional information to empower students' decision-making, engaging students in physical safety planning, and building trust and emotional safety between students and staff. Efforts to implement the most common themes of empowerment, trust and transparency, safety, and collaboration were often interrelated: for example, *transparency* about institutional processes *empowered* students to make the best choice for their situation, and smooth *collaboration* and handoffs between team members increased students' *safety*. Conversely, interviews revealed that some staff were limited in implementing TIA by their federally mandated roles. For example, confidential resources like counselors, advocates, and health practitioners expressed their main priorities of unequivocally believing students, trusting them to make the decisions best for them, and following the student's lead in how they wanted to

move forward. By contrast, Title IX and safety staff had legal obligations to complainants, respondents, and the broader campus community, which precluded them from being centered only on the complainant. These findings align with the quantitative data, where safety staff showed generally less engagement with TIA than health staff, and again suggests IHEs may choose to evaluate how external law enforcement agencies engage in sexual assault response on campus or improve training for this particular sector.

Despite tensions due to differences in roles, participants commented on how they balanced different goals to build an effective, trauma-informed response. They described efforts to understand how each other's roles and responsibilities guided their responses, such as acknowledging that advocates and health staff focus on centering students and respecting their agency, law enforcement may need to apprehend an alleged offender, and Title IX must provide equitable support to both a complainant and respondent. Ultimately, regular collaborative interactions and maintaining respectful relationships with campus colleagues despite different priorities were key to trusting, synergetic, and effective teams that served students' needs well.

## Limitations

The quantitative survey data are subject to three main limitations. First, the coincidence of CSAR survey data collection with administrative challenges related to the COVID-19 pandemic may have suppressed participation among smaller IHEs (those under 1,000 students), which were underrepresented in the final sample. Second, the reliability of survey participants' knowledge—individually and if they chose to respond without consulting with colleagues—may have varied across IHEs. Third, the survey did not gather data about responsibilities of mandatory reporters compared to confidential resources, which became a retrospective limitation after this distinction between staff types appeared prominently in the qualitative findings.

Qualitative data may be impacted by self-selection bias in those campuses that opted to participate, and potential bias in the staff members who were selected to participate in the interviews. While this strategy was necessary to account for differences in which staff are involved in CSAR teams on different campuses, the perspectives of the individuals who participated in this study could differ from those who were not selected, but who are similarly engaged in response efforts (Gibbs et al., 2007; Papoutsakis, 2008). In addition, the different organizational structures within health and safety sectors at the IHEs surveyed and interviewed contributed to the heterogeneity in staff roles represented, which limited the robustness of comparisons across sectors for both the quantitative and qualitative analyses. Similarly, while we aimed to gather a diverse sample of sites (based on size, public or private status, and length of academic programs), the sample is not representative of all institutions of higher education across the U.S. and thus results cannot be widely generalized (Gibbs et al., 2007).

## Study Contributions And Future Directions

From our review of the literature, there are few studies specifically investigating perceptions of how TIA are applied in sexual assault response efforts. The qualitative aspects of this study bring more granularity to IHE staff's trauma-informed approaches, beyond what was captured in the initial survey data. In addition, while TIA are an evidence-based practice and IHEs have embraced it as a concept (Hunter, 2022; Nathan & Ferrara, 2020), few studies focus on testing its efficacy specifically for responding to sexual assault at IHEs. Over half of the 15 studies included in a systematic review of the effectiveness of trauma-informed approaches focused on mental health and child social services environments; none of the studies examined sexual assault responses or IHEs (Fernández et al., 2023). The current study highlights efforts by different sectors and staff roles to practice TIA in response to sexual violence against college students, a key demographic that experiences sexual assault at high rates during an important developmental

timeframe (Koss et al., 2022; Schroeder et al., 2023).

This research also suggests several possible avenues for practice and additional study. First, this study collected limited data about how to incorporate an understanding of cultural and gender issues when implementing a trauma-informed approach. This may be due to a lack of practical direction for how staff can reflect these issues in trauma-informed approaches to everyday response processes. Some literature describes the need for training, education, and evaluation of trauma-informed approaches in IHEs (Henshaw, 2022), but additional study into how IHE administrations can recognize unique experiences of students with multiple, intersecting identities and implement culturally responsive TIA would be helpful for creating robust student services and care. Second, interview participants strongly supported proactively building positive working relationships across different sectors involved in sexual assault response. Though the participants we interviewed generally noted solid collaboration across different sectors on campus, the quantitative data showed disparities in how different staff types embraced TIA in the national sample of IHEs, especially between certain types of safety staff in comparison to health and victim-focused services. Collaboration has been shown to lead to effective multisector partnerships, innovations in service delivery, and can have positive impacts on client and program outcomes (O'Leary & Bingham, 2009; Selden et al., 2006). Thus, results about collaboration are a key takeaway for practitioners aiming to improve their response to sexual misconduct at their IHE. Third, several participants discussed developing their own one-pagers, flow charts, and other resources to help parse the complexities of a survivor's options for recourse and recovery, suggesting that broader dissemination of similar tools could help practitioners across IHEs.

Another prominent takeaway from this study was the difficulty of operating within federal constraints while implementing TIA, and the disconnect between policymakers and the college administrators responsible for carrying out policies on their campuses. Participants described the challenges of weighing mandatory reporting requirements while also attempting to build rapport with students, readjusting campus policies and procedures in light of frequent Title IX regulation changes, and observing how Clery Act compliance conflicted with efforts to center a complainant. These findings were also reflected in our literature review, and speaks to the need for more research and practical guidance for translating laws and policies into practice as IHEs respond to campus sexual assault (Fisher & Cullen, 2013; Gronert, 2019; Streng & Kamimura, 2015; Terman, 2022). Research also suggests that alternative approaches, such as the restorative justice model, may provide a more survivor-centered avenue for dispute resolution beyond federal guidelines (Gronert, 2019; Koss et al., 2014). Beyond these efforts, this study and related research may advise law and policymakers in centering survivors and building trauma-informed approaches to sexual assault as they shape higher education policy.

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Tables and Figures

**Table 1: Defining the SAMHSA Framework and Core Principles**

| **Principle** | **Definition (from SAMHSA's 2014 Concept of Trauma and Guidance for a Trauma-Informed Approach)**[1] |
|---|---|
| Empower, Voice, and Choice | Supporting individuals reporting a sexual assault by giving them choice, making them aware of their options, empowering them to use their strengths, and leveraging a person-centered orientation. |
| Collaboration and Mutuality | Efforts to increase or maintain collaboration across those responding to a reported sexual assault to ensure continuity of care or to remove the burden on individuals involved in the event. |
| Trust and Transparency | Establishing transparency and trust, both between campus staff responding to an incident and between parties involved in the event. |
| Safety | Efforts to make spaces physically, emotionally, and psychologically safe when responding to sexual assault incidents. |
| Peer Support | Efforts to use peer support as a factor for building safety, trust, and collaboration with students involved in a Title IX case. |
| Cultural and Gender Issues | Efforts to move past cultural stereotypes and biases to build a response system that is sensitive to the racial, ethnic, and cultural needs of individuals. |

[1] *SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach*. Substance Abuse and Mental Health Services Administration; 2014. https://store.samhsa.gov/product/SAMHSA-s-Concept-of-Trauma-and-Guidance-for-a-Trauma-Informed-Approach/SMA14-4884

**Table 2: Qualitative Site and Participant Characteristics**

| **Site** | **School Characteristics** | **Number of Participants by Role** |
|---|---|---|
| Campus Size | | |
| Campus Type | | |
| Degree Program | | |
| Health & Wellness | | |
| Safety & Security | | |
| Title IX / Compliance | | |
| Advocacy | | |
| Student Services | | |
| **Total** | | |

**1**
Small
Private
4-year
3
1
2
0
0
**6**
**2**
Large
Private
4-year
1
1
0
1
2
**5**
**3**
Large
Public
2-year
1
2
1
0
1
**5**
**4**
Small
Public
2-year
2
1
1
1
2
**7**
**5**
Small
Public
4-year
2
1
4
0
0
**7**
**6**
Large
Public
4-year
2
1
2

2
0
**7**
**7**
Small
Public, MSI
4-year
2
1
2
0
1
**6**

**Total**

**13**
**8**
**12**
**4**
**6**
**43**

**Figure 1: Proportion of different staff roles with skillsets to respond to trauma related to sexual misconduct.**

Strongly Disagree or Disagree
Neither Agree/Disagree
Strongly Agree or Agree

- Proportions do not add up to 100% due to responses of “don’t know” not shown here.
- Total n=830 IHEs who completed the survey.
- Ns: External Law Enforcement n=107; Institutional Police (sworn) n=442-444; Institutional Public Safety (non-sworn) n=397-398; Contract Security n=206-208; Victim Support n=233-234; Mental Health n=576-579; Health Promotion n=740-742; Health Care n=500-502

Strongly Disagree or Disagree
Neither Agree/Disagree
Strongly Agree or Agree

- Proportions do not add up to 100% due to responses of “don’t know” not shown here.
- Total n=830 IHEs who completed the survey.
- Ns: External Law Enforcement n=107; Institutional Police (sworn) n=442-444; Institutional Public Safety (non-sworn) n=397-398; Contract Security n=206-208; Victim Support n=233-234; Mental Health n=576-579; Health Promotion n=740-742; Health Care n=500-502

**Figure 2: Intersections between principles during implementation.**

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